Vandetanib: RET-positive medullary thyroid cancer

National Cancer Control Programme (NCCP) 00242 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before starting

Validated RET mutation test; full blood count, kidney and liver tests, potassium, calcium, magnesium, thyroid-stimulating hormone, electrocardiogram (ECG) and blood pressure. Baseline calcitonin is additionally advised.

STEP 2

Taking or receiving treatment

Take at the same time daily, with or without food. If a dose is missed, take it when remembered unless less than 12 hours remain before the next dose; then skip it. Never double a dose. If swallowing is difficult, disperse the whole tablet without crushing in half a glass of still drinking water, stir for about 10 minutes and drink immediately; rinse the glass with another half-glass and drink. Use no other liquid. Pharmacy must teach feeding-tube administration if needed.

STEP 3

Tests and reviews

Blood count, kidney and liver tests at weeks 1, 3, 6 and 12. ECG, potassium, calcium, magnesium and thyroid-stimulating hormone at weeks 1, 3, 6 and 12, then every 3 months for at least a year. Repeat this schedule after a QT-related reduction or interruption longer than 2 weeks. Continue checks after stopping when clinically indicated; monitor blood pressure.

STEP 4

Specific practical instructions

Carry the manufacturer's patient alert card and receive one with every prescription. Report fainting or palpitations urgently. Check every new medicine for heart-rhythm interactions. Diarrhoea can disturb blood salts and increase rhythm risk. Protect skin from sunlight.

STEP 5

Leaving with a clear plan

Confirm the next appointment, dose, support medicines and verified daytime and out-of-hours phone numbers.

Sources: NCCP 00242 v6: official regimen. Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending.